

10240 SW 56th ST, Unit 101, Miami, FL 33165

TO: <i>Integrity</i>	FROM: Navarro Medical Centers
FAX: <i>1844-215-4265</i>	FAX: 786-332-4367
PHONE: <i>1844-215-4264</i>	PHONE: 786-332-4577
SUBJECT: <i>Pure WICK at home</i>	SUBJECT: <i>12/19/22</i>
RE: <i>Stella Miller</i>	RE:

URGENT**FOR REVIEW****PLEASE REPLY****PLEASE RECYCLE****STAT****Thank you,
Navarro Medical Staff****HAVE A WONDERFUL DAY!**

Patient Name: Stella Miller
Street Address:
Mailing Address: 14961 Polk ST Miami FL 33176
Home Phone: 305-905-0248
Work Phone: Leave a Message:
Date of Birth: 09/15/1927 Social Security Number:
Marital Status: Unknown Email Address:

Insurance Information

Primary Insurance: Simply Health plan Phone Number:
9250 W FLAGLER ST STE 60, MIAMI, FL, 33174-3415
Subscriber Name: Miller, Stella Date of Birth : 09/15/1927
Subscriber ID: 150W06221 Group Number:

Employer Information

Employer Name: Phone Number:
Address :
Emergency Contact Name: Phone Number:
Pharmacy Name: NAVARRO MEDICAL CENTERS Pharmacy Number:
786-332-4577



10240 SW 56TH ST, Suite 101
Miami, FL 33165
Tel: 7863324577 • Fax: 786-332-4367

Aniel Navarro Marin, MD
Family Medicine
DEA #: FN6320799 • LIC #: FL
NPI #: 1750724621

NONACUTE PAIN ☐

ACUTE PAIN EXCEPTION ☐

Name.

Shelley Miller

DOB

9/15/1927

Address.

14961 York St Miami FL 33176

Date.

R

Pure wick at home

SIGNATURE

[Signature]

DK: R 39.81, L42.8

MD

LICENSURE

LABEL ☐

NO SUBSTITUTION ☐

<i>Refill</i>	NR	1	2	3	4	5
---------------	----	---	---	---	---	---

SAFETY FEATURES: COLORED VOID BACKGROUND • MICROPRINT LINES • IMPRINT ERASURE PROTECTION
REVERSE RX • THERMOCHROMIC INK • ON-BACK: ARTIFICIAL WATERMARK • COIN-REACTIVE INK

FileRx.com 800-307-7717 RxPads.com
Order # 3910696-1
RX_2_Full_H

PYR22682428179

SCRIPT# 7688

16/12/22, 17:01

MILLER, Stella DOB: 09/15/1927 (95 yo F) Acc No. 9452 DOS: 12/06/2022

Patient: Miller, Stella
Account Number: 9452
DOB: 09/15/1927 **Age:** 95 Y **Sex:** Female
Phone: 305-905-0248
Address: 14961 Polk ST, Miami, FL-33176

Provider: Aniel Navarro, MD
Date: 12/06/2022

Subjective:

Chief Complaints:

1. Follow up.

HPI:

New/Follow-up Patient Consult:

95 years old female patient who I visit via telemedicine. The patient is at home. I am in the office. Caregiver gave me verbal consent. She is under hospice care. She has been declining. Family requests a special urine collector. Her daughter spoke with the insurance and she was told that it was going to be approved. This patient is basically dependent for all activities of daily living. She is mostly bedbound. PPS 30%. Decreased appetite.

ROS:

General ROS:

The patient's ROS and history is pertinent for Due to patient clinical condition and cognitive status History provided by Primary Caregiver..

Medical History: Essential (primary) hypertension, Congestive heart failure, unspecified, Rheumatoid arthritis, Essential (primary) hypertension, Valvular cardiomyopathy, Mixed hyperlipidemia, Rheumatoid arthritis involving multiple sites, unspecified rheumatoid factor presence, Primary osteoarthritis of both knees, Major depress, part remis, GAD (generalized anxiety disorder), Longstanding persistent atrial fibrillation, Chronic anticoagulation.

Medications: Taking Oxygen Permeable Lens Products - Solution as directed , Taking Mirtazapine 15 MG Tablet 1 tablet at bedtime Orally Once a day, Taking Sertraline HCl 25 mg Tablet 1 TABLET ONCE A DAY ORALLY 30 DAY(S) , Taking Donepezil HCl 10 MG Tablet 1 tablet at bedtime Orally Once a day, Taking Eliquis 2.5 MG Tablet TAKE 1 TABLET BY MOUTH EVERY 12 HOURS , Taking Bumetanide 1 MG Tablet Take 1 tablet Orally twice a day, Taking amLODIPine Besylate 5 mg Tablet 1 TABLET ONCE A DAY ORALLY 30 DAY(S) , Taking Pantoprazole Sodium 40 MG Tablet Delayed Release 1 tablet Orally Once a day, Taking Valsartan 160 MG Tablet 1 tablet Orally Twice a day

Allergies: N.K.D.A.

Objective:

Vitals:

Telemedicine.

Examination:

General Examination:

GENERAL APPEARANCE: In no acute distress..
HEAD: normocephalic, atraumatic.
EYES: pupils equal, round, reactive to light and accommodation.
EARS: No deformities or erythema of earlobe.
NOSE: No deformities. No nasal discharge.
ORAL CAVITY: mucosa moist.
THROAT: clear.
NECK/THYROID: neck supple, full range of motion..
SKIN: no suspicious lesions.
HEART: no jugular venous distention

Provider: Aniel Navarro, MD

Date: 12/06/2022

MILLER, Stella DOB: 09/15/1927 (95 yo F) Acc No. 9452 DOS: 12/06/2022

LUNGS: No tachypnea, no respiratory distress.

ABDOMEN: Non distended.

RECTAL: Deferred.

BACK: no deformity.

FEMALE GENITOURINARY: Deferred.

MUSCULOSKELETAL: No deformities.

EXTREMITIES: no clubbing, cyanosis.

PERIPHERAL PULSES: Deferred.

NEUROLOGIC: nonfocal, motor strength normal upper and lower extremities, sensory exam intact.

PSYCH: No depression, anxiety, suicidal or homicidal ideation.

Assessment:

Assessment:

1. MDD (major depressive disorder), recurrent episode, moderate - F33.1 (Primary)
 2. Hypertensive heart and chronic kidney disease with heart failure and stage 1 through stage 4 chronic kidney disease, or unspecified chronic kidney disease - I13.0
 3. Stage 3b chronic kidney disease - N18.32
 4. Longstanding persistent atrial fibrillation - I48.11
 5. Chronic anticoagulation - Z79.01
 6. Other thrombophilia - D68.69
 7. Rheumatoid arthritis involving multiple sites, unspecified rheumatoid factor presence - M06.9
 8. Urinary incontinence due to cognitive impairment - R39.81
- 95-years-old female patient on hospice care who has visit via telemedicine. The patient is completely dependent for all activities of daily living. She is mostly bedbound. She is incontinent. The patient is continent in the bowels. She is oriented x1 and confused. PPS 30%. See recommendations below.

Plan:

1. MDD (major depressive disorder), recurrent episode, moderate

Clinical Notes: Stable. Extensive counseling regarding this condition discussed with the patient. Alarm signs for suicidal or homicidal ideations discussed. ED precautions discussed. Continue current plan with sertraline.

2. Hypertensive heart and chronic kidney disease with heart failure and stage 1 through stage 4 chronic kidney disease, or unspecified chronic kidney disease

Clinical Notes: Stable. Continue to control chronic related conditions including Hypertension, Hyperlipidemia, Diabetes. Hydration and low potassium and phosphate diet counseling provided. Monitor every 3 months with labs. Alarm signs and symptoms discussed. ED precautions discussed.

3. Stage 3b chronic kidney disease

Clinical Notes: See above.

4. Longstanding persistent atrial fibrillation

Clinical Notes: normal ventricular response. Continue Anticoagulation and medications to control the rate. Monitor.

5. Chronic anticoagulation

Clinical Notes: for atrial fibrillation. Continue medication. Bleeding signs discussed with caregivers.

6. Other thrombophilia

Clinical Notes: the patient has persistent atrial fibrillation on chronic Anticoagulation. Extensive counseling provided. Continue to monitor.

7. Rheumatoid arthritis involving multiple sites, unspecified rheumatoid factor presence

Clinical Notes: Stable. Continue symptomatic treatment. Patient on hospice services. Monitor.

8. Urinary incontinence due to cognitive impairment

Clinical Notes: especially urine collection device will be requested (PureWickAtHome.com).

9. Others

Clinical Notes: At least 30 minutes of face-to-face encounter was provided to the patient and caregiver. Plan of care discussed in detail with the patient and her caregiver. All questions were answered for the patients and

16/12/22, 17:01

Print Preview

MILLER, Stella DOB: 09/15/1927 (95 yo F) Acc No. 9452 DOS: 12/06/2022

caregiver full satisfaction. Medication reconciliation was done. Extensive counseling was provided. Patient recommended to call us back if any questions or concerns. The patient and caregivers verbalized understanding and agreement with the plan.

Follow Up: 4 Weeks (Reason: Chronic conditions)

Billing Information:

Visit Code:

99213 Office Visit, Est Pt., Level 3.

Procedure Codes:

Electronically signed by ANIEL NAVARRO MARIN , MD on 12/07/2022 at 11:28 AM EST

Sign off status: Completed

Provider: Aniel Navarro, MD

Date: 12/06/2022